

Office of Enforcement and Removal Operations
ICE Health Service Corps

U.S. Department of Homeland Security
500 12th Street, SW
Washington, D.C. 20536



U.S. Immigration
and Customs
Enforcement

February 5, 2021

MEMORANDUM FOR:

(b)(6),(b)(7)(C),(b)(7)(F) DHSc, FACHE
Assistant Director
ICE Health Service Corps

(b)(6),(b)(7)(C),(b)(7)(F)

THROUGH:

(b)(6),(b)(7)(C),(b)(7)(F) MD
Deputy Assistant Director of Clinical Services/Medical Director
ICE Health Service Corps

(b)(6),(b)(7)(C),(b)(7)(F)

(b)(6),(b)(7)(C),(b)(7)(F) MS
Chief of Staff
ICE Health Service Corps

(b)(6),(b)(7)(C),(b)(7)(F)

CDR (b)(6),(b)(7)(C),(b)(7)(F) PA-C
Acting Chief, Investigations Unit
ICE Health Service Corps

(b)(6),(b)(7)(C),(b)(7)(F)

FROM:

CDR (b)(6),(b)(7)(C),(b)(7)(F) RN
Investigator
ICE Health Service Corps

(b)(6),(b)(7)(C),(b)(7)(F)

SUBJECT:

Mortality Review – Report of Findings
Jose Freddy GUILLEN Vega, (b)(6),(b)(7)(C)

Executive Summary:

On August 10, 2020, U.S. Immigration and Customs Enforcement (ICE) Health Service Corps (IHSC) received notification of the death of ICE detainee Jose Freddy GUILLEN Vega, (b)(6),(b)(7)(C). During his detention period, ICE housed Mr. GUILLEN, a 70-year-old Costa Rican male, at the Stewart Detention Center (SDC) in Lumpkin, Georgia (GA), from July 15 to August 10, 2020, the date of his death. Mr. GUILLEN's death certificate noted his cause of death was related to acute respiratory distress syndrome and corona virus disease 2019 (COVID-19).

On August 9, 2008, prior to Mr. GUILLEN's ICE custody and detention, ICE Enforcement and Removal Operations (ERO) Atlanta encountered Mr. GUILLEN at the North Carolina

Mortality Review - Jose Freddy GUILLEN Vega, (b)(6),(b)(7)(C)

Page 2 of 8

Department of Corrections (NCDC) in Marshall, North Carolina (NC), where he was serving a statutory rape conviction and ERO served him an Immigration Detainer, Notice to Action, form I-247. On February 18, 2020, ERO Atlanta encountered Mr. GUILLEN at the Warren Correction Institute (WCI) in Manson, NC, and served a second Immigration Detainer, Notice of Action, form I-247. On July 10, 2020, WCI released Mr. GUILLEN to ERO Atlanta at Alamance County Detention Facility in Graham, NC. ERO Atlanta served Mr. GUILLEN a Final Administrative Removal Order, form I-851, charging removability as an alien convicted of an aggravated felony. On July 15, 2020, ERO Atlanta transferred Mr. GUILLEN to SDC for housing, where he remained for the duration of his ICE custody.

On July 15, 2020 during intake screening, Mr. GUILLEN reported a history of diabetes mellitus [(DM); a chronic condition characterized by high blood sugar, because the body does not produce enough insulin to meet its physiological needs], hypertension [(HTN); high blood pressure], hyperlipidemia (high levels of fat in the blood), chronic kidney disease, hypothyroidism (underactive thyroid gland) managed with medications. A registered nurse (RN) documented a negative coronavirus (COVID-19) symptom screening, normal vital signs (VS) and an elevated blood sugar (BS) of 210 milligrams/deciliter [(mg/dl); normal range 70 - 100 mg/dl]. Upon completion of Mr. GUILLEN's intake screening, SDC medical staff co-horted Mr. GUILLEN for 14 days due to SDC's COVID-19 protocols.

On July 16, 2020, an advanced practice provider (APP) reviewed Mr. GUILLEN's intake screening, acknowledged his medical history, and completed his physical examination. The APP documented exam findings of hemosiderin staining (brownish patches above the ankles), trace pitting edema (swelling) to his lower extremities, an elevated blood pressure (BP) of 171/86 millimeters of mercury [(mmHg); normal range 90/60 – 120/80 mmHg], enrolled him into the chronic care clinic, continued his medications ordered laboratory studies, and an electrocardiogram [(EKG); measures the electrical activity of the heart].

On July 31, 2021, an RN evaluated Mr. GUILLEN during sick call for fatigue, chills, and poor appetite. Mr. GUILLEN denied having a cough, shortness of breath (SOB), nausea, vomiting or loss of smell. The RN observed Mr. GUILLEN as alert and oriented, and documented the following abnormal VS: BP of 150/79 mmHg, temperature (T) of 100.6 degrees (°) Fahrenheit (F) (normal 97.8 – 99.1°F), BS level of 221 mg/dl, and low oxygen saturation [(SpO2): fraction of oxygen saturated hemoglobin in the blood] of 90 percent (%) (normal oxygen saturation 95 – 100%) on room air. The RN notified the APP and received a verbal order to transfer Mr. GUILLEN to Piedmont Columbus Regional Hospital (PCRH) emergency department (ED) in Columbus, GA, via emergency medical services (EMS). The RN activated EMS and continued to monitor Mr. GUILLEN until EMS arrived.

Upon arrival to PCRH, a physician (MD) evaluated Mr. GUILLEN, ordered a chest x-ray (CXR), and laboratory studies, which included a COVID-19 test. Mr. GUILLEN's COVID-19 test and CXR results were positive and showed bilateral infiltrates (substance denser than air),

Mortality Review - Jose Freddy GUILLEN Vega, (b)(6),(b)(7)(C)

Page 3 of 8

respectively. A few hours later, an MD re-assessed Mr. GUILLEN, diagnosed him with COVID-19 related pneumonia, documented SpO2 range of 95 – 96%, ordered amoxicillin and azithromycin (antibiotics), noted Mr. GUILLEN remained hemodynamically stable and discharged him back to SDC.

On this same date, after Mr. GUILLEN's return to SDC, an RN evaluated him and documented normal VS, except for an elevated BP of 141/84 mmHg. The RN notified the MD and received a telephone order to admit Mr. GUILLEN to the medical observation unit for positive COVID-19 test results and monitoring.

On August 1, 2020, an RN evaluated Mr. GUILLEN for weakness, fatigue, decreased appetite, dizziness, nausea with one episode of vomiting, and a low SpO2 level of 86% on room air. The RN notified the MD, received a telephone order to administer supplemental oxygen at three liters per minute (L/m) via nasal canula (NC), re-evaluate Mr. GUILLEN in thirty minutes, and provide the MD with an update of his condition. The RN re-assessed Mr. GUILLEN and noted his oxygen saturation increased to 89 – 90%. The RN notified the MD of Mr. GUILLEN's current status and received a telephone order to transfer Mr. GUILLEN to PCRH ED via EMS. Upon arrival to PCRH, an MD evaluated Mr. GUILLEN, noted decreased SpO2 at 90% on 3 L/m, fever, and an elevated BP of 165/76 mmHg. The MD ordered a CXR and admitted Mr. GUILLEN to the medical-surgical floor for positive COVID-19 and decreased SpO2 levels.

On August 6, 2020, the medical staff increased Mr. GUILLEN's supplemental oxygen to 40 L/m via high flow NC, transferred him to the step down intensive care unit (an intermediate level of care between the intensive care unit (ICU) and the medical-surgical unit) for a higher level of care, and placed him on a bi-level positive airway pressure [(BiPAP); used to increase intrapulmonary pressure to reduce pulmonary vascular congestion] machine.

On August 8, 2020, Mr. GUILLEN developed a right collapsed lung, tachypnea (increased respiratory rate), and tachycardia (increased heart rate). An MD inserted a right chest tube, ordered anti-hypertensive and antiarrhythmic medications, and transferred Mr. GUILLEN to the ICU.

On August 10, 2020, Mr. GUILLEN experienced cardiopulmonary arrest (loss of heart function, breathing, and consciousness) five times and required advanced life-saving measures, with successful resuscitations; however, at approximately 11:27 p.m., Mr. GUILLEN experienced cardiopulmonary arrest again, and the PCRH medical staff initiated advance life-saving measures but were unable to resuscitate him. A PCRH MD pronounced Mr. GUILLEN deceased at 11:37 p.m.

Mortality Finding:

As a result of Mr. GUILLEN's death, the IHSC Assistant Director requested a mortality review

Mortality Review - Jose Freddy GUILLEN Vega, (b)(6),(b)(7)(C)

Page 4 of 8

to learn from his death by reviewing the care provided and the circumstances leading up to his death. The goal of the mortality review is to determine the appropriateness of clinical care; ascertain whether changes to policies, procedures, or practices are warranted; and identify issues that require further study.

A mortality review committee (MRC) determined that Mr. GUILLEN's medical care at SDC was not provided within the safe limits of practice. Specifically, the MRC determined SDC's care, or lack thereof, could pose a future risk. Overall findings identified were: 1) SDC's lack of documentation regarding Mr. GUILLEN's medical condition, while on medical observation; 2) SDC's nursing staff lacked post-hospital orders for Mr. GUILLEN's follow up care, while on medical observation; and 3) SDC's outdated policies regarding COVID-19 and emergency protocols.

The MRC convened on January 21, 2021, and consisted of the following members: Captain (CAPT) (b)(6),(b)(7)(C),(b)(7)(F) MD, Medical Asset Support Team physician, physician reviewer; CAPT (b)(6),(b)(7)(C),(b)(7)(F) Physician Assistant - certified, Eastern Regional APP, APP reviewer; CAPT (b)(6),(b)(7)(C),(b)(7)(F) RN, IHSC Chief Nurse, nurse reviewer; and CAPT (b)(6),(b)(7)(C),(b)(7)(F) Nurse Practitioner – certified, IHSC Deputy Assistant Director of Health Care Compliance (DAD of HCC), compliance reviewer.

The following summary of events was based on: 1) Mr. GUILLEN's SDC and PCRH medical records, and EMS reports; 2) incident and notification reports; 3) ICE ENFORCE Alien Removal Module (EARM) and ICE ENFORCE Alien Detention Module (EADM) database records; 4) Mr. GUILLEN's SDC detention file; 5) a remote review and virtual staff interviews conducted by Fact-Finder Commander (b)(6),(b)(7)(C),(b)(7)(F) on October 26 – 27, 2020; and 6) applicable SDC policies and procedures, and ICE Detention Standards.

Summary of Events

On July 15, 2020, an RN completed Mr. GUILLEN's intake screening, documented a negative COVID-19 symptom screening, normal VS, and an elevated BS of 210 mg/dl. The RN reviewed Mr. GUILLEN's transfer summary, and noted a medical history of DM, HTN, hyperlipidemia, chronic kidney disease, and hypothyroidism and the following medications: atorvastatin (treats high cholesterol and triglyceride levels) 40 milligrams (mg), one tablet by mouth every evening, enalapril (anti-hypertensive) 10 mg, one tablet by mouth daily, hydrochlorothiazide (treats high blood pressure and fluid retention) 25 mg, one tablet by mouth daily, levothyroxine (treats low thyroid levels or enlarged thyroid gland) 75 micrograms (mcg), one tablet by mouth daily, metformin (anti-diabetic medication) 500 mg by mouth daily, metoprolol (anti-hypertensive) 50 mg, one tablet by mouth two times daily, and pioglitazone (anti-diabetic medication) 30 mg, one tablet by mouth daily.

On July 16, 2020, an APP completed Mr. GUILLEN's physical examination, reviewed his intake

Mortality Review - Jose Freddy GUILLEN Vega, (b)(6),(b)(7)(C)

Page 5 of 8

screening, acknowledged his medical history, documented additional medical findings of hemosiderin staining and trace pitting edema to his lower extremities. The APP noted Mr. GUILLEN's VS as normal, except for an elevated BP of 171/86 mmHg, and enrolled him into the chronic care clinic. The APP continued Mr. GUILLEN's medications for his DM, HTN, hyperlipidemia, hypothyroidism, ordered Novolin Regular insulin (used for DM management) per sliding scale coverage, laboratory studies to assess his kidney function, and an EKG.

On July 31, 2020 at approximately 1:05 p.m., an RN evaluated Mr. GUILLEN during sick call for complaints of fatigue, lack of appetite, chills for two days, and documented the following VS: T of 100.6, BP of 150/79 mmHg, pulse (P) of 83 bpm (normal range 80 – 100 bpm), respiratory rate (RR) of 18 breaths per minute (bpm) (normal range 12 – 20 bpm), and SpO2 of 90% on room air. The RN referred him to an APP for further evaluation. The APP evaluated Mr. GUILLEN, consulted with the MD, administered acetaminophen (used to relieve pain and reduce fever) 1000 mg by mouth, and gave a verbal order to transfer Mr. GUILLEN to PCRH via EMS for a higher level of care.

- At approximately 3:46 p.m., Mr. GUILLEN arrived at PCRH ED. An MD evaluated Mr. GUILLEN, ordered a CXR, and laboratory studies, which included a COVID-19 test. The COVID-19 test was positive, and the CXR results showed bilateral infiltrates.
- At approximately 7:25 p.m., an MD reassessed Mr. GUILLEN, diagnosed him with COVID-19 related pneumonia, documented SpO2 range of 95 – 96%, ordered amoxicillin and azithromycin, noted Mr. GUILLEN remained hemodynamically stable and discharged him back to SDC.
- At approximately 11:12 p.m., Mr. GUILLEN returned to SDC. An RN evaluated Mr. GUILLEN and documented normal VS, except for an elevated BP of 141/84 mmHg. The RN notified the MD and received a telephone order to admit Mr. GUILLEN to the medical observation unit for positive COVID-19 test results and monitoring.

On August 1, 2020 at approximately 9:55 a.m., an RN evaluated Mr. GUILLEN for weakness, fatigue, decreased appetite, dizziness, nausea with one episode of vomiting, and a low SpO2 level of 86%, on room air. Mr. GUILLEN denied having SOB, chest pain, headache, cough or fever. The RN notified the MD, received a verbal order to administer supplemental oxygen at 3 L/m via NC, re-evaluate Mr. GUILLEN in thirty minutes and provide an update of his condition. Mr. GUILLEN's oxygen saturation increased to 89 - 91%.

- At approximately 10:30 a.m., the RN notified the MD, provided an update of Mr. GUILLEN's status and SpO2 levels. The MD gave a verbal order to transfer Mr. GUILLEN to PCRH ED via EMS.
- At approximately 3:59 p.m., an ED MD evaluated and admitted Mr. GUILLEN to PCRH with a low-grade fever of 99.3°F, elevated BP of 165/76 mmHg, and low SpO2 of 90% on 3 L/m via NC. The MD ordered laboratory studies, a CXR, supplemental oxygen, plasma transfusion, and intravenous antibiotics.

Mortality Review - Jose Freddy GUILLEN Vega, (b)(6),(b)(7)(C)

Page 6 of 8

On August 2, 2020, an infectious disease (ID) specialist evaluated Mr. GUILLEN, noted Mr. GUILLEN was on supplemental oxygen at 6 L/m via NC with an SpO2 of 90 -92%, and elevated inflammatory markers (used to monitor inflammatory conditions) with acute kidney injury. The ID specialist ordered a five-day course of intravenous remdesivir (anti-viral medication).

On August 6, 2020, the medical staff increased Mr. GUILLEN's oxygen supplementation to 40 L/m via high flow NC with an SpO2 of 90%, transferred him to the step-down ICU for a higher level of care and placed him on a BiPAP machine.

On August 8, 2020, Mr. GUILLEN presented with an elevated P of 130 - 140 bpm, RR rate of 32 bpm, and decreased BP and breath sounds to his right lung. An MD ordered antihypertensive and antiarrhythmic medication, and a CXR, which showed a collapsed right lung. The MD inserted a right-sided chest tube and transferred Mr. GUILLEN to the ICU.

On August 10, 2020, from approximately 9:44 a.m. to 5:29 p.m., Mr. GUILLEN experienced cardiopulmonary arrest five times and required advanced life-saving measures, with successful resuscitation.

- At approximately 11:27 p.m., Mr. GUILLEN experienced cardiopulmonary arrest again, and the PCRH medical staff initiated advance life-saving measures; however, their efforts were unsuccessful. A PCRH MD pronounced Mr. GUILLEN deceased at 11:37 a.m., confirmed with an echocardiogram (a test that uses ultrasound to show the heart beating and pumping blood).

Mr. GUILLEN's death certificate noted the immediate cause of death was related to acute respiratory distress syndrome and COVID-19, with contributory causes of the death of DM, HTN, and chronic kidney disease. The coroner determined the manner of death as natural.

Detention Standards used for this review: ICE Performance-Based National Detention Standards (PBNDS), 2011 (revised December 2016), and the National Commission on Correctional Health Care Standards (NCCHC), 2018.

The following is a summary of health care delivery/program strengths, weaknesses, and recommendations found during this review:

Strengths

Strengths identified during this investigation were: 1) SDC medical staff's early recognition of Mr. GUILLEN's symptoms and prompt referral to a higher level of care; 2) SDC's MD's active involvement in Mr. GUILLEN's care while hospitalized; and 3) in early August 2020, SDC conducted facility-wide COVID-19 testing; thereafter, a process was initiated to test all new arrivals and cohort for seven to ten days.

Mortality Review - Jose Freddy GUILLEN Vega, (b)(6),(b)(7)(C)

Page 7 of 8

Weaknesses

1. Documentation.

- On July 31, 2020, SDC medical staff placed Mr. GUILLEN on medical observation due to positive COVID-19 results and for continuous monitoring of his condition; however, the medical staff did not document on his medical status until the next morning.

Applicable standards of care for this finding:

- NCCHC 2018 standards, J-A-08, *Health Records*.

2. Patient Care.

- On July 31, 2020,
 - SDC medical staff placed Mr. GUILLEN on medical observation due to positive COVID results and to monitor his condition; however, the staff did not evaluate him until the next morning.
 - Mr. GUILLEN presented with fatigue, lack of appetite, chills and a low SpO2 level of 90%; however, the SDC nursing staff did not administer supplemental oxygen prior to his transfer to PCRH.
 - There is no documentary evidence regarding MD orders for medical care during Mr. GUILLEN's placement on medical observation.
- Several medical staff reported lack of an after-action discussion to evaluate the medical staff's observations and response to Mr. GUILLEN's symptoms.

Applicable standards of care for this finding:

- PBNDS 2011: 4.3, Medical Care; section T. *Emergency Medical Services and First Aid*.
- NCCHC 2018: J-B-08, *Patient Safety*.
- NCCHC 2018: J-D-07, *Emergency Services and Response Plan*.
- SDC Policy: 13-71, *Physician Orders and Patient Care Protocols*.

3. Policies.

- Some of SDC's COVID related policies and guidance were not dated, which could potentially lead to staff not using the most updated guidance.
- SDC does not have a formalized emergency protocol to standardize treatment for low SpO2 levels in patients with suspected or possible COVID-19.

Applicable standards of care for this finding:

Mortality Review - Jose Freddy GUILLEN Vega, (b)(6),(b)(7)(C)

Page 8 of 8

- NCCHC 2018: J-A-05, *Policies and Procedures*.
- SDC Policy: 13-71, *Physician Orders and Patient Care Protocols*, November 2018.
- SDC Policy: *COVID-19 Guidance for Correctional and Detention Facilities*.
- SDC Policy: *Medical Emergency Pandemic Coronavirus (COVID-19) Plan*.

Recommendations

- Forward these findings to the IHSC Deputy Assistant Director (DAD) of Health Care Compliance (HCC).
- The IHSC DAD of HCC will share these findings through appropriate communication channels to ICE, the SDC administrator and health authority for review and any action(s) deemed appropriate.
- The respective IHSC HCC Unit and ICE will ensure a corrective action plan (CAP) is implemented and sustained.

End of report.